

Antibacterial

CUI MYS-003
R.O.C. Reg. No.:031835

Ceflour

Injection 0.75g



Description:

A white or faintly yellow powder in vial.

Ingredient:

Each vial contains:

Cefuroxime (as Sodium) 0.75g (potency)

Pharmacodynamics:

- a) Cefuroxime has in vitro activity against a wide range of gram-positive organisms, and it is highly stable in the presence of β -lactamases of certain gram-negative bacteria. It is more active against many of the Enterobacteriaceae including some strain of Enterobacter, E. coli, Klebsiella and Salmonella spp. and indolepositive Proteus spp.
- b) Cefuroxime's bacterial action results from inhibition of cell-wall synthesis.

Pharmacokinetics:

- a) Cefuroxime is widely distributed in the body including pleural fluid, sputum, bone, synovial fluid and aqueous humour.
Therapeutic serum concentrations of approximately 2mg/L or more were maintained from 5.3 hours and 8 hours or more, respectively.
- b) Approximately 89% of a dose of Cefuroxime is excreted by the kidneys over an 8 hour period, resulting in high urinary concentrations.
- c) Cefuroxime is approximately 50% bound to serum protein.

Indications:

Respiratory tract infections: acute and chronic bronchitis, infected bronchiectasis, bacterial pneumonia, lung abscess and post-operative chest infections.
Ear, nose and throat infections: sinusitis, tonsillitis, pharyngitis and otitis media.
Urinary tract infections: acute and chronic pyelonephritis, cystitis and asymptomatic bacteriuria.
Soft tissue infections: cellulitis, erysipelas and wound infection.
Bone and joint infections: osteomyelitis and septic arthritis.
Obstetric and gynaecological infections: pelvic inflammatory disease.
Gonorrhoea particularly when penicillin is unsuitable.
Other infections including septicaemia, meningitis and peritonitis.
Prophylaxis against infection in abdominal, pelvic, orthopedic, cardiac, pulmonary, oesophageal and vascular surgery where there is increase risk from infections.

Dosage and Administration:

I.M. /I.V. injection:

a. Adult:

1. Usual dose, 0.75g every 8 hours.
2. For uncomplicated urinary tract infection, skin and skin structure infection, disseminated gonococcal infections, and uncomplicated pneumonia, a 0.75g dose every 8 hours is recommended.
3. In severe or complicated infections, a 1.5g dose every 8 hours is recommended. In life threatening infections or infections due to less susceptible organisms, 1.5g every 6 hours may be required.
4. For bacterial meningitis, the dose should not exceed 3g every 8 hours.
5. The recommended dose for uncomplicated gonococcal infection is 1.5g intramuscularly, given as a single dose at two different sites together with 1.0g of oral probenecid.
6. For surgical infection prophylaxis, a 1.5g dose administered intravenously just prior to surgery followed by a 0.75g intravenously or intramuscularly every 8 hours.
7. For prophylaxis during an open heart surgery, a 1.5g dose administered intravenously at the induction of anesthesia and every 12 hours thereafter for a total of 6g is recommended.

b. Child dose:

1. Infants and children above 3 months of age- 50 to 100 mg/kg/day in equally divided doses every 6 to 8 hours.
For more severe infections, a higher dose of 100 mg/kg/day (not to exceed maximum adult dose) may be used.
2. For bacterial meningitis, larger doses are recommended, initially 200 to 240 mg/kg/day intravenously in divided doses every 6 to 8 hours.

c. Impaired renal function:

Creatinine clearance (mL/min)	Dose	Frequency
>20	750mg-1.5g	Every 8 hours
10-20	750mg	Every 12 hours
<10	750mg	Every 24 hours

d. Preparation of solution and suspension

1. For intramuscular use:
Reconstitute 0.75g Cefuroxime with 3.6 mL of Sterile Water for injection. Shake gently and withdraw 3.6 mL of the resulting suspension for injection.
2. For intravenous use:
Reconstitute 0.75g of Cefuroxime with 8.0 mL of Sterile Water for injection. Withdraw 8.0 mL of the resulting solution for injection.
3. For infusion:
Reconstitute 0.75g or 1.5g dose of Cefuroxime with 50 to 100 mL of Sterile Water for injection, 5% Dextrose injection or 0.9% Sodium Chloride injection. If Sterile Water for injection is used as the diluent, reconstitute with 20ml/g to avoid a hypotonic solution.
To be used only by a physician.

Contraindications:

Contraindicated in patients with known allergy to the cephalosporin group of antibiotics.

Precautions:

1. Cefuroxime should be given with caution to patients receiving concurrent treatment with potent diuretics and to patients with renal impairment.
2. The total daily dose of Cefuroxime should be reduced in patients with impaired renal function.
3. Cefuroxime should be given with caution in patients with a history of gastrointestinal disease, particularly colitis.
4. As with other antibiotics, prolonged use of Cefuroxime may result in overgrowth of nonsusceptible organisms. Careful observation of the patient is essential.
5. There are no adequate and well-controlled studies in pregnant woman. This drug should be used during pregnancy only if clearly needed.
6. Caution should be exercised when Cefuroxime is administered to a nursing woman as it is excreted in human milk.
7. Safety and effectiveness in children below 3 months of age have not been established. Prolongation of drug half-life of other members of the cephalosporin class in newborn infants has been reported.
8. As with other cephalosporin, Ceflour powder as well as solutions and suspensions tend to darken, depending on storage conditions, without adversely affecting product potency.

Warning:

BEFORE THERAPY WITH CEFUROXIME IS INSTITUTED, CAREFUL INQUIRY SHOULD BE MADE TO DETERMINE WHETHER THE PATIENT HAS HAD PREVIOUS HYPERSENSITIVITY REACTIONS TO CEFUROXIME, CEPHALOSPORINS, PENICILLINS, OR OTHER DRUGS. IF THIS PRODUCT IS TO BE GIVEN TO PENICILLIN SENSITIVE PATIENTS, CAUTION SHOULD BE EXERCISED BECAUSE CROSS-HYPERSENSITIVITY INCLUDING ANAPHYLAXIS, AMONG BETA-LACTAM ANTIBIOTICS HAS BEEN CLEARLY DOCUMENTED.

If an allergic reaction to Cefuroxime occurs, the drug should be discontinued and if necessary, the patient should be treated with appropriate agents, eg, pressor amines, antihistamines or corticosteroids. Antibiotics, including Cefuroxime should be administered cautiously to any patient who has demonstrated some form of allergy, particularly to drugs. Pseudomembranous colitis has been reported with virtually all broad-spectrum antibiotics (including macrolides, semisynthetic penicillins and cephalosporins); therefore, it is important to consider its diagnosis in patients who develop diarrhea in association with the use of antibiotics. Such colitis may range in severity from mild to life threatening. Mild cases of pseudomembranous colitis usually respond to drug discontinuance alone. In moderate to severe cases, appropriate measures should be taken.

Pregnancy and Lactation:

- a) There are no adequate and well-controlled studies in pregnant women. This drug should be used during pregnancy only if clearly needed.
- b) Caution should be exercised when Cefuroxime is administered to a nursing woman as it is excreted in human milk.

Adverse Reactions:

Transient pain at the injection site following I.M. administration and thrombophlebitis following I.V. infusion may occur.
Other adverse effects that may occur include allergic reactions, including skin rashes, urticaria, eosinophilia, fever, and anaphylaxis.

Interactions:

1. Cefuroxime may cause false-negative reactions in the ferricyanide test for blood glucose.
2. Cefuroxime may also cause false-positive result in the Coombs'(antiglobulin) test.
3. Cefuroxime does not interfere with the assay of serum and urine creatinine by the alkaline picrate method.
4. Concurrent use of anticoagulants, heparin or thrombolytic agents with cefuroxime may increase the risk of severe hemorrhage. All cephalosporins can inhibit Vitamin K synthesis by suppressing gut flora. Prophylactic Vitamin K therapy is recommended when cephalosporin is used for prolonged periods in malnourished or seriously ill patients.
5. Hypoprothrombinemia induced by large doses of salicylates and/or cephalosporins and the gastrointestinal ulcerative or haemorrhagic potential of nonsteroidal anti-inflammatory drugs (NSAIDS), salicylates, or sulfapyrazone may increase the risk of haemorrhage.
6. Probenecid decreases renal tubular secretion of cephalosporins excreted by this mechanism, resulting in increased and prolonged cephalosporin serum concentrations, prolonged elimination half-life, and increased risk of nephrotoxicity. However, this combination may be used concurrently in the treatment of sexually transmitted diseases (STDs) or other infections, in which high and/or prolonged antibiotic serum and tissue concentrations are required.

Overdosage:

Overdosage of cefuroxime can cause cerebral irritation leading to convulsions. Other than general supportive treatment, no specific antidote is known. Excessive serum levels of cefuroxime can be reduced by dialysis.

Shelf-Life:

3 years from the date of manufacture.

Storage Condition(s):

Prior to reconstitution:

Store at temperature below 30°C . Protect from light.

Reconstituted solutions may be stored for up to 24 hours at room temperature or 48 hours under refrigeration (at or below 5°C). Unused solutions should be discarded after the time periods mentioned above.

Packagings:

0.75g x 50Vials/Box.

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Manufacturer:

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